

Annex

Here is the requested content to support the **MDT Integrated Rehabilitation Model**:

1. Detailed SOP Templates

Each SOP follows a standard, practical format suitable for a Vietnamese social protection center (low-resource, training-ready, easy to translate/adapt). You can copy these into Word/Excel for formatting with tables, headers, and approval signatures.

SOP-04: Daily Implementation & Integration Protocol

(Highest priority – most used in daily operations)

SOP Number: SOP-04

Version: 1.0

Effective Date: [Insert]

Approved by: Center Director / System Architect

Review Date: Every 6 months or after major changes

Purpose

To ensure rehabilitation goals from the Integrated Individual Plan (IIP) are consistently embedded into daily care routines and classroom activities, maximizing functional practice opportunities for PWD.

Scope

Applies to all caregivers, teachers, and support staff working with PWD who have an active IIP at Kon Tum Social Work Center.

Responsibilities

- **Caregivers/Teachers:** Responsible for embedding and documenting rehab actions during routines/activities.
- **Case Manager (Social Worker):** Monitors compliance through logs and weekly reviews.

- **Rehabilitation Therapist:** Provides guidance/techniques and periodic training.
- **System Architect:** Ensures adherence and addresses barriers.

Procedure (Step-by-Step)

1. At the start of shift/day, review the PWD's current IIP (Sections 3–4) for relevant goals.
2. Identify integration opportunities in scheduled routines (meals, dressing, hygiene) or learning activities (writing, group work, physical play).
3. Embed the rehab action using guided support (do not fully perform the task for the PWD unless safety requires it). Use simple prompts/scripts provided in training.
4. During or immediately after the activity, complete the **Daily Integration Log** (see template below).
5. Note any challenges, successes, or need for plan adjustment.
6. Submit completed log to Case Manager by 4:00 PM daily.

Key Behavioral Rules

- Every activity must link to at least one IIP goal.
- Prioritize PWD choice and active participation.
- Caregivers act as “co-therapists.”

Forms/Templates

Daily Integration Log Template (one row per activity or per PWD):

Date	PWD Name/ID	Routine/Activity	Linked IIP Goal (SMART)	Embedded Rehab Action Performed	Progress/Observation (e.g., independent with verbal cue / no physical assist)
		Eating lunch	Self-feed full meal using adapted spoon within 4 weeks	Guided hand-over-hand for grip strengthening	Completed with minimal physical assist and spilled less than yesterday

Key Performance Indicators (KPIs)

- 100% of daily routines/activities for pilot PWD have documented integration.
- ≥80% of logs show clear linkage to IIP goals.

Training Requirements

- Initial 2-hour training on Goal Linking + hands-on practice.
- Refresher every 3 months or when new IIP is introduced.

References

- IIP Template (SOP-03)
 - Integration Matrices (from Implementation Plan Section 5)
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SOP-05: Weekly MDT Coordination Meeting Protocol

SOP Number: SOP-05

Purpose

To ensure regular, data-driven review and adjustment of the IIP through interdisciplinary collaboration.

Scope

All core MDT members (Therapist, Teacher, Caregiver representative, Case Manager).

Procedure

1. Case Manager circulates agenda + latest logs/IIP updates 24 hours in advance.
2. Meeting duration: 45–60 minutes (fixed time, e.g., every Friday 9:00 AM).
3. Standard Agenda:
 - a. Therapist presents functional progress first.
 - b. Teacher reports education integration.
 - c. Caregiver shares daily observations/ADL progress.
 - d. Review barriers and family feedback.
 - e. Adjust IIP goals/interventions if needed (document in Section 6).
 - f. Assign follow-up actions with deadlines.

4. Case Manager records minutes and distributes within 24 hours.
5. Update IIP and communicate changes to all staff.

Templates

- Meeting Agenda Template
- MDT Meeting Minutes Template (with columns for Progress, Barriers, Adjustments, Responsible Person, Deadline)

KPIs

- 100% attendance of core MDT.
 - All discussions data-backed (logs/assessments).
 - Adjustments made when <70% progress on a goal.
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SOP-06: Monitoring & Daily Logging

Purpose

To standardize collection of progress data for timely decision-making.

Key Elements

- Use the Daily Integration Log (from SOP-04).
- Weekly summary by Case Manager.
- Simple observation scale: Independent / Minimal Assist / Moderate Assist / Maximal Assist / Not Achieved.

Other SOPs (SOP-01 Intake, SOP-02 Assessment, SOP-03 IIP Development, SOP-07 Review & Update, SOP-08 Quality Assurance) follow the same structure:

Purpose, Scope, Responsibilities, Procedure, Forms, KPIs, Training, References.

You can expand the full package by duplicating this format for each.

Recommended Annex Tools

- Quick-reference laminated cards with integration matrices and example prompts.
- Digital or paper log book (one per PWD or shared binder).

2. Sample Filled Integrated Individual Plan (IIP)

IIP Template – Sample Filled Version

(Fictional but realistic case – suitable for pilot testing)

Section 1: Basic Information

- PWD Name/ID: Nguyễn Thị Lan / PWD-KT-045
- Date of Birth: 15/03/2015 (Age 11)
- Diagnosis: Cerebral palsy (spastic diplegia) with mild intellectual disability – Agent Orange-related family history
- Assigned Case Manager: Social Worker – Trần Thị Mai
- IIP Start Date: 01/06/2026
- Next Review: 29/06/2026 (4 weeks)

Section 2: Multidisciplinary Assessment Summary (brief)

- Therapist: Limited independent walking (max 3m with walker), weak grip, good motivation.
- Teacher: Short attention span (5–8 min), enjoys drawing, needs support in group activities.
- Caregiver: Requires full assist for dressing/hygiene; family reports fatigue.
- Social Worker/Family Voice: Lan wants to “eat by myself like my sister” and “play with friends without falling.” Family priority: increase independence to reduce caregiver burden.

Section 3: Goal Setting (SMART)

Functional Goals

1. Lan will walk 8 meters indoors using a posterior walker with standby assistance only, in under 2 minutes, 3 times per day (by 30/06/2026).
2. Lan will independently self-feed 80% of a meal using adapted spoon and non-slip plate, with minimal verbal cues (by 15/07/2026).

Education Goals (linked)

3. During writing/drawing activities, Lan will maintain tripod grasp and complete 10 letter strokes with minimal hand-over-hand assistance (linked to fine motor goal, by 30/06/2026).

4. In group work, Lan will take turns and use simple gestures/words to request item from peer in 4 out of 5 opportunities (linked to social participation, by 15/07/2026).

Daily Living Goals

5. Lan will participate in dressing upper body by pulling on shirt with verbal and minimal physical prompts (by 30/06/2026).

Section 4: Intervention Plan & Role Assignment

Goal #	Intervention/Embedded Action	Frequency/Setting	Responsible	Support Needed
1	Balance & gait training during hygiene (weight shift) + short walks to classroom	Daily routines + 2x/week technical session	Caregiver + Therapist	Walker, verbal cues
2	Hand coordination during meals (guided grip)	Every meal	Caregiver (co-therapist)	Adapted utensils
3	Fine motor practice in art class	Daily classroom	Teacher + Therapist guidance	Thick crayons, adaptive grips
4	Social cues in group play	Daily group time	Teacher	Visual schedule

Section 5: Daily Integration Log (example for one week – summarized)

[Insert table similar to SOP-04 log with 5–7 entries showing progress, e.g., “Self-feeding: 60% independent on Day 3”]

Section 6: Weekly MDT Review Notes

- Week 1 Progress: Goal 2 at 50% – good motivation but fatigue in afternoons.
- Adjustments: Add shorter, more frequent practice sessions after rest.
- Barriers: Caregiver shortage on weekends – solution: train family visitor.

Section 7: Family/Caregiver Capacity & Support Plan

- Family strengths: Mother motivated, visits twice weekly.

- Support: Weekly 30-min family training session on guided support techniques. Provide simple home adaptation list (e.g., non-slip mats).
- Caregiver burnout risk: Rotate duties; offer peer support group.

Section 8: Outcome Measurement & Signatures

- Measurement Tools: Simple observation scale + parent feedback.
- Signatures:
 - PWD (or mark): Lan (with assistance)
 - Family: Nguyễn Thị Hương (Mother) – Date: 01/06/2026
 - Case Manager: Trần Thị Mai
 - Therapist / Teacher / Caregiver: [Signatures]

This sample IIP is **person-centered**, uses **SMART goals**, demonstrates **goal linking**, and includes **family voice** — exactly as recommended in the enhanced Implementation Plan.

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Here are the additional practical materials to complete the **MDT Integrated Rehabilitation Model** toolkit for the Kon Tum Social Work Center consultancy. These align directly with the enhanced Implementation Plan and TOR requirements (training-ready materials, special education integration, and operational usability).

1. Training Curriculum Outline

This **5-module curriculum** is designed for a mixed group of staff (therapists, teachers, caregivers, social workers) at a provincial social protection center. It emphasizes hands-on, low-resource delivery suitable for Vietnam (2–3 days initial training + ongoing refreshers). Total initial training: ~16–20 hours, delivered in Vietnamese with practical simulations using real or pilot cases.

Overall Objectives

By the end of training, participants will be able to:

- Explain the MDT model and “One PWD = One Plan” principle.
- Develop and use SMART functional goals with goal linking.
- Embed rehabilitation into daily care and education routines.

- Participate effectively in weekly MDT meetings and use SOPs.
- Engage families as partners while managing caregiver burden.

Target Audience: Core MDT staff (10–20 participants).

Methods: Lectures (20%), group discussions, role-plays, case simulations, hands-on practice with IIP templates and logs.

Trainers: System Architect + external consultant + senior center staff (train-the-trainer approach).

Materials Needed: Projector, printed SOPs/IIP templates, daily log forms, role-play scenarios, laminated integration matrices.

Module 1: Introduction to the MDT Integrated Model (2–3 hours)

- Background: Problems with silos vs. benefits of integrated, person-centered care.
- Core principles (person-centered, function-oriented, integration-focused, interdisciplinary).
- Roles & responsibilities (RACI matrix), Case Manager as linchpin, System Architect support.
- Vietnamese context: Alignment with disability laws, family dynamics, resource realities.
- Activity: Group mapping of current vs. ideal workflows.

Module 2: Person-Centered Assessment and IIP Development (4 hours)

- 360-degree multidisciplinary assessment with family voice.
- IIP structure and “single source of truth” concept.
- Writing **SMART goals** and goal linking across disciplines.
- Family visioning and co-production techniques.
- Activity: Complete a sample IIP using a real/pilot case (physical or intellectual disability profile).

Module 3: Integration into Daily Care and Special Education (5–6 hours)

- Philosophy: Rehabilitation happens in the other 23 hours.
- Daily care and education integration matrices.
- Practical embedding techniques and behavioral mandates.
- Caregivers as “co-therapists”; teachers linking learning to functional skills.

- Activity: Role-plays of mealtime, dressing, writing/drawing, and group activities with guided support scripts.

Module 4: SOPs, Monitoring, and MDT Coordination (4 hours)

- Overview of the 8 SOPs with focus on SOP-04 (Daily Integration), SOP-05 (Weekly Meetings), and SOP-06 (Logging).
- Using daily logs, meeting agendas, and progress tracking.
- KPIs and simple outcome measurement.
- Activity: Simulate a full weekly MDT meeting using sample logs and IIP data.

Module 5: Implementation, Quality Assurance, and Sustainability (3–4 hours)

- Change management, addressing resistance and staff turnover.
- Family engagement and caregiver support plans.
- Risk mitigation, pilot checklist, and post-pilot mentoring.
- Train-the-trainer: Participants prepare to onboard new staff.
- Evaluation: Pre/post knowledge quiz + confidence self-assessment.
- Next steps: 3-month supervision plan and refresher schedule.

Evaluation & Follow-up

- Pre/post training survey on knowledge and skills.
- Observation during pilot simulations.
- Quarterly refresher sessions (2–4 hours) + on-the-job mentoring by Case Manager/System Architect.
- Certification of completion for staff records.

This curriculum is practical and scalable. It can be adapted into a full training manual with slides, handouts, and video demonstrations of integration techniques.

2. Sample SMART Goals and Integration Scripts

SMART Goals Examples (tailored to common profiles in Agent Orange-affected areas, e.g., cerebral palsy/spastic diplegia or intellectual disabilities). All goals are functional, measurable, and linked across disciplines.

Mobility / Gross Motor

- Lan (11 years, spastic diplegia) will walk 8 meters indoors using a posterior walker with standby assistance only (no hands-on help) in under 2 minutes, 3 times per day during transitions to classroom or meals, by 30/06/2026.

(Linked to: Education – moving to group activities; Care – hygiene transfers)

Self-Feeding / Fine Motor & ADL

- Lan will independently self-feed 80% of a meal using an adapted spoon and non-slip plate with minimal verbal cues only, during all lunch and dinner times, achieving this in 4 out of 5 meals per week by 15/07/2026.

(Linked to: Education – hand strength for writing/drawing)

Dressing / Sequencing & Balance

- Minh (9 years, mild intellectual disability + motor delay) will independently pull on an upper-body shirt with verbal prompts only and maintain balance while standing, during morning dressing routine, 5 out of 7 days per week by 30/06/2026.

(Linked to: Therapy – balance training; Caregiver – reducing full assistance)

Social Participation / Communication

- During group learning activities, Lan will take turns and use simple words or gestures to request an item from a peer in 4 out of 5 opportunities, with visual cue card support, across 3 group sessions per week by 15/07/2026.

(Linked to: Teacher – social skills in classroom; Therapist – communication cues)

Fine Motor in Education

- During art/writing class, Minh will maintain a functional tripod grasp and complete 10 letter strokes or drawings with minimal hand-over-hand assistance, in 4 out of 5 daily sessions by end of June 2026.

(Linked to: Therapy – grip strengthening during meals/hygiene)

Tips for Writing SMART Goals: Make them Specific (who/what/where), Measurable (how much/how often), Achievable (based on baseline), Relevant (to PWD/family priorities), Time-bound (4–8 weeks for short-term).

Integration Scripts / Prompts (simple, consistent language for caregivers and teachers – use in daily routines)

During Mealtime (Self-Feeding Goal)

- “Lan, hold the spoon like this [model grip]. You can do it – scoop and bring to mouth. I’ll help only if you need.” (Verbal cue)
- “Great job using your hand! One more bite by yourself.” (Positive reinforcement)
- If struggling: Hand-over-hand guidance → fade to wrist support → verbal only.

During Dressing (Upper Body)

- “First the head, then the arms. Pull down slowly – use your balance.” (Sequence prompt)
- “You’re standing strong! Now the other arm.” (Encourage weight shift)
- “Good try! Next time you do more by yourself.” (Encourage autonomy)

During Writing/Drawing Class (Fine Motor Link)

- “Hold the crayon with three fingers like when you eat. Press gently and make the line.” (Link to meal goal)
- “Excellent control! This helps your hand get stronger for feeding too.” (Explicit goal linking)

During Hygiene / Toileting (Balance & Mobility)

- “Shift your weight to one leg while washing – just like walking practice.”
- “You’re doing the steps yourself today – I’m right here if you need me.” (Guided support, not full assistance)

General Staff Prompt

- “Remember the IIP goal for this activity? How can we practice it here?” (Before starting routine)

These scripts promote **guided participation** rather than doing tasks for the PWD, reducing caregiver burden while building independence.

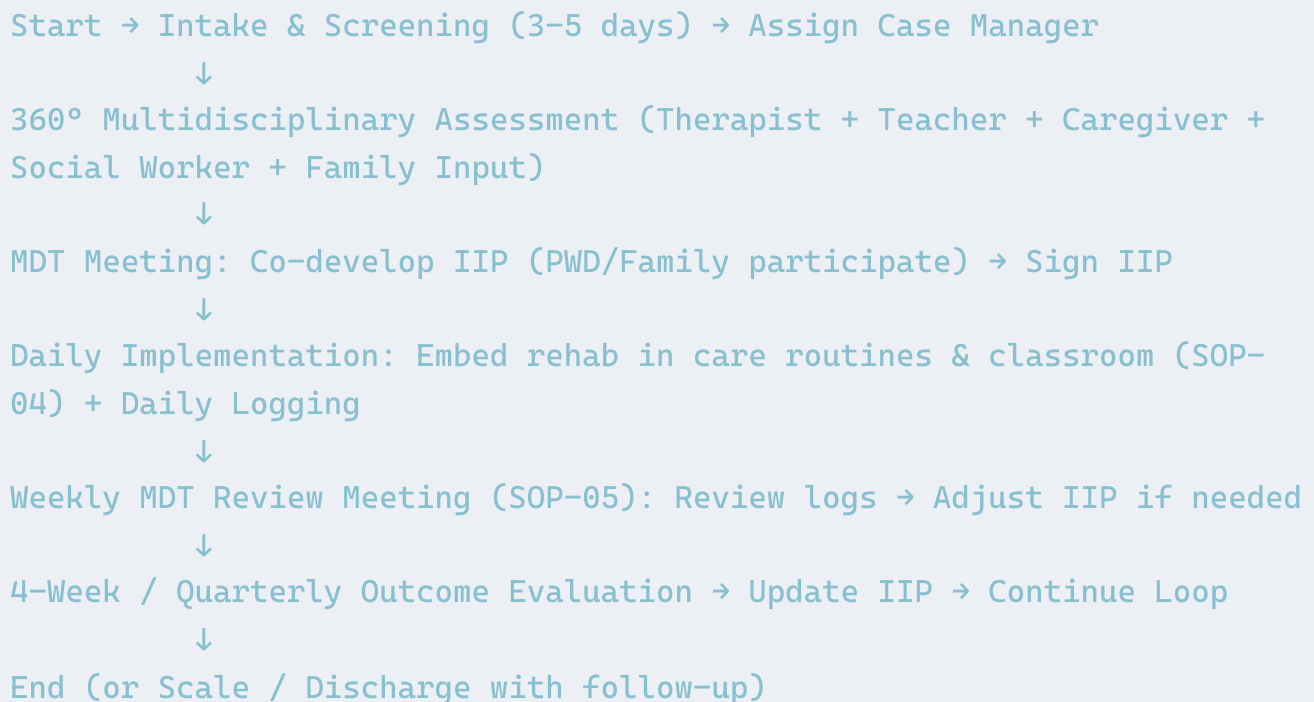
3. Visual Diagrams and Flowcharts (Text-Based Descriptions)

These can be easily created in Microsoft Visio, Lucidchart, PowerPoint, or Canva for the final deliverables. Below are ready-to-use textual descriptions and simple ASCII/Markdown representations.

A. MDT Organigram (Coordination Structure)

- Top: Center Director / Management
- Middle: System Architect (Process support, reports to Director)
- Core MDT Circle (interconnected):
 - Social Worker (Case Manager) – central node, owns IIP
 - Rehabilitation Therapist
 - Teacher (Special Education)
 - Caregivers (rotating representative)
- Arrows: Bidirectional coordination loops + weekly meeting link.
- Extended MDT (on side): Speech therapist, psychologist, etc., consulted as needed.
- Family/PWD connected via Case Manager.

B. Service Pathway Flowchart (Main visual for TOR Deliverable 1)



Key Decision Points: If progress <70% → intensify embedding or modify goal.
Barriers identified → family support plan activated.

C. IIP Development Process Flowchart

1. Assessment → Summarize findings
2. Family Visioning Session → Prioritize goals
3. MDT Goal Setting & Linking → Draft Sections 3–4
4. Review & Sign (PWD/Family + MDT) → Distribute copies
5. Integrate into daily logs & routines

D. Integration Opportunity Matrix (Visual Job Aid)

A simple table (as in the Implementation Plan) can be turned into a colorful poster with icons:

- Columns: Routine/Activity | Embedded Action | Functional Target | Staff Prompt
Example

These visuals make the model easy to understand and reference daily. Recommend including them as annexes or wall charts in the center.

These materials — together with the previous Implementation Plan, SOP templates, and sample IIP — form a complete, coherent, and immediately usable technical package for the TOR deliverables (MDT model by 30/5/2026 and SOP package by 15/6/2026).